NUR 774 Exam 3: 100-Question Practice Exam

Respiratory, ear, eye, cardiovascular, medications, and immunizations. Built from your Integrated Study Guide. Choose the ONE best answer. Answer key with explanations starts after question 100. Aim for 80%+ (80/100).

1. An 8-year-old has symptoms 3 days/week, wakes at night 3 times/month, uses albuterol 3 days/week, has minor activity limitation, and an FEV1 of 85%. Classify the severity:

A. Severe persistent
B. Intermittent
C. Moderate persistent
D. Mild persistent

2. A child has sharp chest pain that worsens lying flat and improves leaning forward. A friction rub is heard. The nurse suspects:

A. Pericarditis
B. Costochondritis
C. Myocarditis
D. Aortic stenosis

3. During a well-child exam, the nurse notes a white pupillary reflex in one eye. What is the priority?

A. Reassure the family because the child has no pain
B. Recheck at the next well visit
C. Prompt ophthalmology referral to evaluate for retinoblastoma
D. Start antibiotic eye drops

4. Which test confirms the location and size of a suspected VSD?

A. Chest x-ray
B. ECG
C. Echocardiogram with Doppler
D. Pulse oximetry

5. A child is nearsighted (myopic). Which statement is correct?

A. The eyes are misaligned
B. Near objects are blurry because light focuses behind the retina
C. Both eyes have unequal refractive errors
D. Distant objects are blurry because light focuses in front of the retina

6. A 9-year-old has daytime symptoms 2 days/week, but wakes at night twice a week. The FEV1 is 82%. Classify the severity:

A. Intermittent
B. Mild persistent
C. Moderate persistent
D. Severe persistent

7. Which murmur characteristic is always considered pathologic?

A. Grade 2 intensity
B. Changes with position
C. Diastolic timing
D. Musical quality

8. A child gets a household cleaner splashed in the eye. What is the first action?

A. Check visual acuity before anything else
B. Begin immediate, copious irrigation
C. Apply an eye patch
D. Take a full history first

9. A 4-year-old with nonsevere AOM is prescribed amoxicillin. What is the typical duration?

A. 5 days
B. 7 days
C. 10 days
D. 14 days

10. A 4-month-old has bronchiolitis with wheeze, mild retractions, an SpO2 of 94%, and is feeding adequately. Which intervention is appropriate?

A. Gentle nasal suction, hydration support, and monitoring
B. Amoxicillin
C. Oral prednisolone
D. Scheduled albuterol nebulizers

11. A teen faints while playing basketball. A systolic ejection murmur is heard. What is the most appropriate response?

A. Reassure, since syncope is common in teens
B. Clear for sports with more hydration
C. Recheck at next year's physical
D. Hold sports clearance and arrange a cardiac evaluation

12. A 7-year-old has sudden sore throat, fever of 39.1°C, tender anterior cervical nodes, and palatal petechiae. No cough or runny nose. The rapid strep test is negative. What should the nurse anticipate next?

A. Send a backup throat culture
B. Reassure the family that the illness is viral
C. Draw an ASO titer to confirm strep
D. Start amoxicillin without further testing

13. A 3-year-old has nonsevere AOM in one ear, mild pain for less than 48 hours, and a temperature of 38.2°C. Which option is appropriate?

A. Pain control with observation and follow-up in 48–72 hours, or antibiotics, by shared decision
B. Withhold pain medication to monitor symptoms
C. Immediate ENT referral
D. Antibiotics are mandatory

14. A 4-year-old has abrupt high fever, drooling, a muffled voice, and is sitting in a tripod position without a barking cough. The nurse's priority is to:

A. Send the child to radiology alone for a neck x-ray
B. Keep the child calm with the parent and prepare for emergency airway care
C. Inspect the throat with a tongue blade
D. Obtain a throat culture

15. A 3-day-old has profuse purulent eye discharge with marked swelling (chemosis). The nurse suspects:

A. Gonococcal conjunctivitis needing urgent systemic ceftriaxone
B. Chlamydial conjunctivitis treated with topical ointment
C. Blocked tear duct
D. Allergic conjunctivitis

16. A toddler with presumed croup has high fever, looks toxic, and does not improve with dexamethasone and epinephrine. The nurse suspects:

A. Bacterial tracheitis
B. Mild viral croup
C. Bronchiolitis
D. Allergic rhinitis

17. Which findings are classic for a significant patent ductus arteriosus?

A. Boot-shaped heart on x-ray
B. Continuous machinery murmur with bounding pulses
C. Weak femoral pulses
D. Fixed split S2

18. A 6-year-old has fever and abdominal pain with a benign abdominal exam. What should the nurse do next?

A. Give ibuprofen and discharge
B. Arrange an abdominal ultrasound
C. Reassure the family that it is a stomach virus
D. Assess respiratory rate and auscultate the lungs

19. A 2-year-old started coughing suddenly while eating peanuts. Now there is wheezing and decreased breath sounds on the right only. The chest x-ray is normal. The next step is:

A. An albuterol trial for new asthma
B. Reassurance because the x-ray is normal
C. Prompt specialist referral for bronchoscopy
D. Amoxicillin for pneumonia

20. A 15-month-old has nonsevere AOM in BOTH ears with a temperature of 38.3°C. The recommended plan is:

A. Antibiotics
B. Ear drops only
C. Observation for 48–72 hours
D. Referral for ear tubes

21. A 16-year-old has fever, exudative pharyngitis, marked fatigue, posterior cervical nodes, and splenomegaly. What is the priority teaching?

A. Avoid contact sports until the spleen is evaluated
B. Stay home from school for 21 days
C. Expect a tonsillectomy
D. Take amoxicillin for 10 days

22. A child being treated for AOM develops fever, redness and swelling behind the ear, and an ear that sticks out. The nurse suspects:

A. Normal healing
B. Otitis media with effusion
C. Mastoiditis requiring urgent evaluation
D. Otitis externa

23. A child with pertussis started azithromycin. When may the child return to school?

A. When the cough completely stops
B. After 5 full days of effective antibiotics
C. After 24 hours of antibiotics
D. After 10 days of antibiotics

24. A 20-kg child took prednisone 2 mg/kg/day for 3 weeks and stopped yesterday. MMR is due. The nurse should:

A. Wait at least 1 month after stopping before giving MMR
B. Never give MMR to this child
C. Give MMR at half dose
D. Give MMR today

25. A 15-year-old has severe throat pain worse on the right, a muffled "hot potato" voice, trismus, and a uvula deviated to the LEFT. What is the most likely diagnosis?

A. Viral pharyngitis
B. Right peritonsillar abscess
C. Epiglottitis
D. Left peritonsillar abscess

26. A tall teen with long fingers, pectus excavatum, and a dislocated lens likely has Marfan syndrome. Which cardiac concern is the priority?

A. Tetralogy of Fallot
B. Aortic root dilation and dissection
C. Coarctation
D. Coronary aneurysm

27. A 6-week-old has episodes of apnea and turning blue after coughing, but no whoop. An older sibling has had a cough for 3 weeks. The most likely diagnosis is:

A. Pertussis
B. Epiglottitis
C. Croup
D. Allergic rhinitis

28. After 72 hours of amoxicillin for pneumonia, a child is still febrile and slightly worse. The best nursing action is to:

A. Continue the same plan for 7 more days
B. Report for reassessment of complications, adherence, dose, or another diagnosis
C. Add an over-the-counter cough suppressant
D. Switch to a nebulized bronchodilator

29. The long-term cardiac damage from rheumatic fever most often affects the:

A. Coronary arteries
B. Mitral (and aortic) valves
C. Pericardium
D. Conduction system only

30. Which is NOT a component of Tetralogy of Fallot?

A. Atrial septal defect
B. Overriding aorta
C. Right ventricular hypertrophy
D. Pulmonary stenosis

31. Which teaching about montelukast is most important?

A. Use it for acute attacks
B. Report mood or behavior changes, including suicidal thoughts
C. It replaces the albuterol inhaler
D. Rinse the mouth after each dose

32. Which instruction is correct for a child's nosebleed?

A. Pinch for 1 minute, then check
B. Lie flat with ice on the forehead
C. Tilt the head back and pinch the bridge
D. Sit up, lean forward, and pinch the soft part of the nose for 10 minutes

33. A 3-year-old received DTaP doses 1 and 2 as an infant, then missed a year. What is the correct plan?

A. No more doses are needed
B. Restart the series from dose 1
C. Continue with dose 3; do not restart the series
D. Give Tdap instead

34. A child has a higher blood pressure in the arms than in the legs, with weak, delayed femoral pulses. The nurse suspects:

A. Patent ductus arteriosus
B. Atrial septal defect
C. Pulmonary stenosis
D. Coarctation of the aorta

35. What is the first-line amoxicillin dose for AOM?

A. 50 mg/kg once daily
B. 20 mg/kg/dose BID
C. 25 mg/kg/day divided TID
D. 80–90 mg/kg/day divided BID

36. An infant with TOF suddenly becomes deeply cyanotic and breathes rapidly while crying. The nurse should first:

A. Place the infant in a knee-chest position
B. Lay the infant flat and supine
C. Start a feeding to calm the infant
D. Give albuterol

37. Which heart sound is the classic finding in an atrial septal defect?

A. Continuous machinery murmur
B. Diastolic murmur at the apex
C. Wide, fixed split S2
D. Harsh holosystolic murmur at the lower left sternal border

38. Why is aspirin avoided as a routine fever reducer in children with viral illness?

A. It causes bronchospasm in all children
B. It is ineffective for fever
C. Risk of Reye syndrome
D. It causes kidney stones

39. A 4-year-old reads the 20/50 line with each eye. What should the nurse do?

A. Refer, because the passing threshold at age 4 is 20/40
B. Pass the child, because 20/50 is normal at 4
C. Refer only if the child has symptoms
D. Retest in 2 years

40. A child is newly diagnosed with mild persistent asthma. Which daily controller medication does the nurse expect?

A. Daily oral prednisone
B. Albuterol as needed only
C. Salmeterol alone
D. Low-dose inhaled corticosteroid

41. A teen's murmur gets LOUDER when standing and during Valsalva. This suggests:

A. Venous hum
B. Still's murmur
C. Hypertrophic cardiomyopathy
D. Normal physiologic split

42. A 30-kg child with confirmed GAS cannot reliably take oral medication. What is the correct single IM dose of benzathine penicillin G?

A. 2.4 million units
B. 600,000 units
C. 1.2 million units
D. 300,000 units

43. A child with croup and stridor at rest receives nebulized epinephrine. What should the nurse plan next?

A. Start scheduled albuterol
B. Give antibiotics
C. Observe for 2–4 hours for return of symptoms
D. Discharge home once stridor resolves

44. An 11-year-old has 10 days of gradually worsening headache, malaise, and dry cough but looks well. Which medication does the nurse anticipate?

A. Amoxicillin
B. Oseltamivir
C. Azithromycin
D. Cephalexin

45. The most serious complication of Kawasaki disease is:

A. Coronary artery aneurysm
B. Hearing loss
C. Kidney failure
D. Mitral valve damage

46. A child's personal best peak flow is 300 L/min. Today's reading is 180 L/min. Which zone is this?

A. Yellow zone
B. Green zone
C. Cannot be determined
D. Red zone

47. Which otoscopic finding best supports a diagnosis of acute otitis media?

A. A red TM in a crying child
B. Clear fluid behind the TM without symptoms
C. Ear tugging
D. Moderate to severe bulging of the tympanic membrane

48. Which finding is an early sign of heart failure in an infant?

A. Marked ankle edema
B. Weight gain from overeating
C. Bradycardia
D. Sweating and tiring during feeds, with poor weight gain

49. A 6-week-old is afebrile with a staccato cough and tachypnea. The infant had conjunctivitis at 2 weeks of age. The most likely organism is:

A. Group B Streptococcus
B. Chlamydia trachomatis
C. Staphylococcus aureus
D. Mycoplasma pneumoniae

50. A parent asks why their child must finish all 10 days of amoxicillin for strep throat when they already feel better. The best response is that completing the course:

A. Is needed only if the child has scarlet fever
B. Prevents poststreptococcal glomerulonephritis
C. Prevents infectious mononucleosis
D. Reduces the risk of acute rheumatic fever

51. A 9-year-old had a sore throat 3 weeks ago and now has migratory knee and ankle arthritis plus a new murmur. ASO is elevated. The most likely diagnosis is:

A. Septic arthritis
B. Kawasaki disease
C. Acute rheumatic fever
D. PSGN

52. A 2-year-old has a barking cough, hoarse voice, and inspiratory stridor only when crying. Symptoms are worse at night. What does the nurse anticipate?

A. Amoxicillin
B. A routine neck x-ray
C. A single dose of oral dexamethasone
D. Albuterol nebulizer

53. A child who developed severe pneumonia shortly after influenza is most likely infected with which organism, in addition to pneumococcus?

A. Group B Streptococcus
B. Staphylococcus aureus, including MRSA
C. Chlamydia trachomatis
D. Mycoplasma pneumoniae

54. An infant received MMR at 9 months before international travel. What is the plan?

A. Still give 2 routine MMR doses starting at 12 months
B. Give one more dose at 4–6 years only
C. No further MMR doses are needed
D. The travel dose counts as dose 1

55. Which finding distinguishes orbital cellulitis from preseptal cellulitis?

A. Red, swollen eyelid
B. Recent sinus infection
C. Pain with eye movement and proptosis
D. Fever

56. A harsh holosystolic murmur at the lower left sternal border is most consistent with:

A. Patent ductus arteriosus
B. Still's murmur
C. Ventricular septal defect
D. Atrial septal defect

57. A 3-year-old has had fever for 6 days, both eyes red without discharge, a strawberry tongue with cracked lips, swollen red hands, and a rash. The nurse suspects:

A. Rheumatic fever
B. Scarlet fever
C. Bacterial conjunctivitis
D. Kawasaki disease

58. A toddler wakes with thick yellow discharge and eyelids glued shut. What else should the nurse assess?

A. Visual acuity with a Snellen chart
B. The ears, for concurrent otitis media
C. Lung sounds for wheezing
D. The skin for a sandpaper rash

59. A murmur is loud and accompanied by a palpable thrill. What is the Levine grade?

A. Grade II
B. Grade III
C. Grade VI
D. Grade IV

60. A child with chickenpox may return to school when:

A. 5 days have passed since the rash appeared
B. No new lesions appear for 2 hours
C. The fever has been gone for 24 hours
D. All lesions are crusted over

61. A child has palpitations every day. Which test is most appropriate?

A. Echocardiogram only
B. Holter monitor
C. Event monitor for several months
D. Cardiac catheterization

62. Which vaccines are routinely given at 11–12 years?

A. MMR and varicella
B. Hib and PCV
C. DTaP, IPV, and MMR
D. Tdap, MenACWY, and HPV

63. A child received IVIG 2 g/kg for Kawasaki disease. How long should MMR and varicella be delayed?

A. 11 months
B. They are permanently contraindicated
C. 1 month
D. 6 months

64. A child has had a firm, painless eyelid nodule for 3 weeks. The most likely diagnosis and initial care are:
A. Chalazion; squeeze to drain it
B. Chalazion; warm compresses and lid hygiene
C. Hordeolum; oral antibiotics
D. Orbital cellulitis; IV antibiotics

65. Which vaccines are routinely given at the 2-month visit?
A. DTaP, IPV, Hib, PCV, and rotavirus
B. Influenza and MMR
C. Tdap, MenACWY, and HPV
D. MMR, varicella, and HepA

66. Which congenital heart defect is CYANOTIC?
A. Atrial septal defect
B. Tetralogy of Fallot
C. Patent ductus arteriosus
D. Ventricular septal defect

67. A 25-kg child with GAS is ordered cephalexin 20 mg/kg/dose BID. What is the correct amount?
A. 500 mg per dose, twice daily
B. 250 mg per dose, twice daily
C. 1,000 mg per dose
D. 500 mg per day total

68. Two weeks after a strep infection, a child has tea-colored urine, puffy eyes, and an elevated blood pressure. The nurse suspects:
A. Kawasaki disease
B. Peritonsillar abscess
C. Acute rheumatic fever
D. Poststreptococcal glomerulonephritis

69. A 5-year-old is entering kindergarten. Which boosters are due at 4–6 years?
A. DTaP, IPV, MMR, and varicella
B. Tdap, HPV, MenACWY
C. MenB and influenza only
D. HepB, Hib, PCV

70. A healthy 12-month-old lives with a grandparent receiving chemotherapy. Regarding MMR and varicella, the nurse should:
A. Withhold both until the grandparent finishes chemo
B. Give both vaccines as scheduled
C. Give MMR only
D. Give varicella only

71. Which child needs dental endocarditis prophylaxis for a qualifying procedure?
A. A child with a small isolated VSD
B. A child with a prosthetic heart valve
C. A child with mitral valve prolapse alone
D. A child with an uncomplicated ASD

72. What causes functional closure of the foramen ovale after birth?

A. Decreased systemic resistance
B. Increased pulmonary vascular resistance
C. Increased left atrial pressure
D. Prostaglandin E1 release

73. Which is an example of CONDUCTIVE hearing loss?

A. Congenital CMV damage to hair cells
B. Hearing loss from middle-ear fluid (OME)
C. Damage to the cochlea
D. Auditory nerve damage

74. A fully immunized 4-year-old has abrupt high fever, tachypnea, and focal crackles in the right lower lobe. The child is stable for outpatient care. The expected treatment is:

A. No antibiotics, supportive care only
B. High-dose amoxicillin, about 90 mg/kg/day divided BID
C. Azithromycin alone
D. Daily IM ceftriaxone for 10 days

75. In fetal circulation, the foramen ovale shunts blood from:

A. Right atrium to left atrium
B. Umbilical vein past the liver
C. Left ventricle to right ventricle
D. Pulmonary artery to aorta

76. Ear irrigation is CONTRAINDICATED for which foreign body?

A. A small plastic bead with an intact TM
B. A button battery
C. Hard cerumen with an intact TM
D. A small stone with an intact TM

77. A healthy 5-year-old has a soft, grade 2, musical systolic murmur that is louder lying down and softer standing. Growth is normal. This is most likely:

A. Ventricular septal defect
B. Hypertrophic cardiomyopathy
C. Aortic stenosis
D. Still's (innocent) murmur

78. Which instruction is correct for a child using an inhaled corticosteroid?

A. Rinse the mouth and spit after each use
B. Stop it once symptoms improve
C. Use it only during asthma attacks
D. Use a spacer with the dry-powder inhaler

79. Which live vaccines are first given routinely at 12–15 months?

A. Rotavirus and HepB
B. MMR and varicella
C. Tdap and HPV
D. DTaP and IPV

80. A child has intensely itchy eyes in both eyes, with watery, stringy discharge, during spring. What should the nurse teach?

A. This is not contagious; use cool compresses and allergy eye drops
B. Start antibiotic ointment
C. Stay home until 24 hours of antibiotic drops
D. Wear an eye patch

81. A child has migratory polyarthritis, fever, and elevated ESR. Why is arthralgia NOT counted as a minor criterion here?

A. Arthralgia requires an ASO titer
B. Arthralgia only counts in adults
C. Arthralgia is a major criterion
D. Arthralgia cannot be counted when arthritis is already a major criterion

82. After a cardiac catheterization through the femoral artery, which assessment is the priority?

A. Urine specific gravity
B. Pupil reactivity
C. Access-site bleeding and pulses distal to the site
D. Bowel sounds

83. A 12-month-old has a mild cold (afebrile) and is taking amoxicillin for AOM. Vaccines are due. The nurse should:

A. Give only non-live vaccines
B. Defer all live vaccines for 1 month
C. Give the scheduled vaccines today
D. Defer until the antibiotic course is done

84. A child weighs 22 kg. Acetaminophen is ordered at 15 mg/kg/dose. The suspension is 160 mg/5 mL. How many mL per dose?

A. 6.9 mL
B. 10.3 mL
C. 15 mL
D. 3.3 mL

85. A 9-year-old has a cough after a cold, scattered rhonchi, no focal findings, and yellow sputum. The appropriate management is:

A. Amoxicillin for 10 days
B. Chest x-ray and CBC before deciding
C. Azithromycin for 5 days
D. Supportive care; antibiotics are not routinely indicated

86. Which medication may be used to CLOSE a PDA in a selected premature infant?

A. Indomethacin or ibuprofen
B. Prostaglandin E1
C. Digoxin
D. Dexamethasone

87. A 44-lb child with AOM is ordered amoxicillin 90 mg/kg/day divided BID. How many mg per dose?

A. 1,980 mg
B. 1,800 mg
C. 450 mg
D. 900 mg

88. A 12-year-old got HPV dose 1, and dose 2 was given 4 months later. What is needed?

A. Another dose, because doses less than 5 months apart are not valid for the 2-dose series
B. Two more doses
C. Restart the series
D. Nothing; the series is complete

89. An 18-kg child is ordered ibuprofen 10 mg/kg/dose. The suspension is 100 mg/5 mL. How many mL?

A. 18 mL
B. 4.5 mL
C. 9 mL
D. 3.6 mL

90. A 2-year-old has AOM along with purulent conjunctivitis. Which antibiotic is preferred?

A. Amoxicillin
B. Topical eye drops only
C. Azithromycin
D. Amoxicillin-clavulanate

91. Coughing fits, an inspiratory whoop, and vomiting after coughing describe which stage of pertussis?

A. Incubation
B. Catarrhal
C. Paroxysmal
D. Convalescent

92. During an asthma exacerbation, a child's wheezing becomes quieter, but the child is more drowsy with poor air movement. The nurse recognizes this as:

A. Improvement from the bronchodilator
B. Normal fatigue after treatment
C. A sign the attack is ending
D. Impending respiratory failure

93. A 16-week-old infant has never received rotavirus vaccine. What should the nurse do?

A. Give a double dose to catch up
B. Do not start rotavirus; the first dose must be given by 14 weeks 6 days
C. Give rotavirus dose 1 today
D. Wait until 6 months to start

94. A child received MMR. Two weeks later, varicella vaccine was given. How should the nurse interpret the varicella dose?

A. It is valid because MMR and varicella are different viruses
B. Only the MMR needs to be repeated
C. It is not valid; injectable live vaccines must be given the same day or at least 28 days apart
D. It is valid; live vaccines can be given at any interval

95. An infant with a large VSD had a normal newborn exam but develops tachypnea and poor feeding at 6 weeks. Why?

A. The ductus arteriosus reopened
B. Pulmonary vascular resistance fell, increasing left-to-right shunting
C. The VSD grew larger
D. The shunt reversed to right-to-left

96. A newborn with d-transposition of the great arteries is severely cyanotic. Which medication does the nurse anticipate?

A. Aspirin
B. Furosemide
C. Indomethacin to close the ductus
D. Prostaglandin E1 to keep the ductus open

97. Which treatment does the nurse expect for Kawasaki disease?

A. Benzathine penicillin monthly
B. Amoxicillin plus ibuprofen
C. Oral prednisone only
D. IVIG 2 g/kg plus aspirin

98. Which presentation meets the criteria for acute bacterial sinusitis?

A. Green nasal mucus for 3 days
B. Sneezing, itchy eyes, and clear drainage
C. Daytime cough and nasal drainage for more than 10 days without improvement
D. Clear rhinorrhea for 5 days with low-grade fever

99. Which group of findings most strongly suggests a VIRAL sore throat rather than group A strep?

A. Palatal petechiae and headache
B. Tender anterior cervical lymph nodes
C. Tonsillar exudate and fever
D. Cough, rhinorrhea, hoarseness, and oral ulcers

100. A child has amber fluid with an air-fluid level behind the TM, no pain, and no fever. The best plan is:

A. Amoxicillin for 10 days
B. Decongestants and antihistamines
C. Watchful waiting for about 3 months, with a hearing check if it persists
D. Immediate ear tube placement

Answer Key and Explanations

1-D **2**-A **3**-C **4**-C **5**-D **6**-C **7**-C **8**-B **9**-B **10**-A **11**-D **12**-A **13**-A **14**-B **15**-A **16**-A **17**-B **18**-D **19**-C **20**-A **21**-A **22**-C **23**-B **24**-A **25**-B **26**-B **27**-A **28**-B **29**-B **30**-A **31**-B **32**-D **33**-C **34**-D **35**-D **36**-A **37**-C **38**-C **39**-A **40**-D **41**-C **42**-C **43**-C **44**-C **45**-A **46**-A **47**-D **48**-D **49**-B **50**-D **51**-C **52**-C **53**-B **54**-A **55**-C **56**-C **57**-D **58**-B **59**-D **60**-D **61**-B **62**-D **63**-A **64**-B **65**-A **66**-B **67**-A **68**-D **69**-A **70**-B **71**-B **72**-C **73**-B **74**-B **75**-A **76**-B **77**-D **78**-A **79**-B **80**-A **81**-D **82**-C **83**-C **84**-B **85**-D **86**-A **87**-D **88**-A **89**-C **90**-D **91**-C **92**-D **93**-B **94**-C **95**-B **96**-D **97**-D **98**-C **99**-D **100**-C

1. D: Mild persistent *[Asthma]*
More than 2 days/week but not daily, and 3–4 nights/month = mild persistent.

2. A: Pericarditis *[Other cardiac]*
Pericarditis = "praying position" relief. Assess for effusion/tamponade.

3. C: Prompt ophthalmology referral to evaluate for retinoblastoma *[Vision]*
Leukocoria = retinoblastoma (or cataract) until proven otherwise. Absence of pain is not reassuring.

4. C: Echocardiogram with Doppler *[Cardiac tests]*
"Echo = Eyes (anatomy); ECG = Electricity." An ECG cannot show a hole.

5. D: Distant objects are blurry because light focuses in front of the retina *[Vision]*
"MY-opia: I only see what's near MY face." Hyperopia = farsighted; anisometropia = unequal; strabismus = misaligned.

6. C: Moderate persistent *[Asthma]*
"Worst feature wins." Night waking more than once a week (but not nightly) = moderate persistent.

7. C: Diastolic timing *[Murmurs]*
"Diastolic is always Dangerous." Exception to know: a venous hum is continuous and innocent.

8. B: Begin immediate, copious irrigation *[Vision]*
Chemical injury: irrigate FIRST. Do not delay irrigation for a history.

9. B: 7 days *[Otitis media]*
"10–7–5": under 2 or severe = 10 days; age 2–5 = 7 days; age 6 and older = 5–7 days.

10. A: Gentle nasal suction, hydration support, and monitoring *[Other respiratory]*
Bronchiolitis = 3 S's: Suction, Sips, Saturation. No routine albuterol, steroids, or antibiotics.

11. D: Hold sports clearance and arrange a cardiac evaluation *[PDA/obstructive]*
"Faints while running = Referral." Consider aortic stenosis or HCM.

12. A: Send a backup throat culture *[Strep pharyngitis]*
A negative rapid antigen test in a symptomatic child aged 3 or older needs a backup throat culture. ASO shows past strep, not acute infection.

13. A: Pain control with observation and follow-up in 48–72 hours, or antibiotics, by shared decision *[Otitis media]*
Age 2+ and nonsevere → observation or antibiotics. ALWAYS treat the pain.

14. B: Keep the child calm with the parent and prepare for emergency airway care *[Croup]*
Epiglottitis = 4 D's (Drooling, Dysphagia, Dysphonia, Distress). Do not inspect the throat. Airway comes first.

15. A: Gonococcal conjunctivitis needing urgent systemic ceftriaxone *[Vision]*
"Gonorrhea Goes fast" (days 2–5); it threatens the cornea. "Chlamydia Comes later" (days 5–12, oral treatment).

16. A: Bacterial tracheitis *[Croup]*
High fever, toxicity, and poor response to standard croup treatment suggest bacterial tracheitis.

17. B: Continuous machinery murmur with bounding pulses *[PDA/obstructive]*
PDA = "machinery." Wide pulse pressure and bounding pulses.

18. D: Assess respiratory rate and auscultate the lungs *[Pneumonia]*
Lower-lobe pneumonia can present as belly pain. Always examine the lungs in a febrile child with abdominal complaints.

19. C: Prompt specialist referral for bronchoscopy *[Foreign body]*
"Sudden + one Side" = foreign body. Food is radiolucent, so a normal x-ray does not rule it out.

20. A: Antibiotics *[Otitis media]*
Under 2 years with both ears affected → antibiotics, even when nonsevere.

21. A: Avoid contact sports until the spleen is evaluated *[Strep complications]*
"Front nodes = Strep, Back nodes = Mono." An enlarged spleen can rupture, so avoid contact sports.

22. C: Mastoiditis requiring urgent evaluation *[Otitis media]*
Postauricular swelling and a protruding pinna = mastoiditis → urgent hospital/ENT care.

23. B: After 5 full days of effective antibiotics *[Pertussis]*
"5 days on drugs, 21 days without" (untreated: 21 days after cough onset).

24. A: Wait at least 1 month after stopping before giving MMR *[Immunizations]*
Steroid "2–20–14": 2 mg/kg/day or 20 mg/day or more for 14 days or more → delay live vaccines at least 1 month.

25. B: Right peritonsillar abscess *[Strep complications]*
The uvula runs AWAY from the abscess. Deviation to the left means the abscess is on the right.

26. B: Aortic root dilation and dissection *[Other cardiac]*
Marfan → aortic root aneurysm/dissection, plus possible MVP.

27. A: Pertussis *[Pertussis]*
Infants with pertussis often have apnea and cyanosis WITHOUT a whoop. Look for a coughing contact.

28. B: Report for reassessment of complications, adherence, dose, or another diagnosis *[Pneumonia]*
No improvement in 48–72 hours → reassess for empyema/effusion, the wrong organism, adherence, or a foreign body.

29. B: Mitral (and aortic) valves *[Rheumatic fever]*
Rheumatic = Valves. Kawasaki = Coronaries.

30. A: Atrial septal defect *[Cyanotic CHD]*
TOF = PROV: Pulmonary stenosis, RVH, Overriding aorta, VSD (ventricular, not atrial).

31. B: Report mood or behavior changes, including suicidal thoughts *[Asthma]*
"Montelukast = Mood warning" (boxed warning). It is not a rescue medication.

32. D: Sit up, lean forward, and pinch the soft part of the nose for 10 minutes *[School and teaching]*
Lean FORWARD and pinch the soft alae continuously for at least 10 minutes. Never tilt back.

33. C: Continue with dose 3; do not restart the series *[Immunizations]*
Catch-up: NEVER restart a series. DTaP is used under 7 years.

34. D: Coarctation of the aorta *[PDA/obstructive]*
Coarctation = "Strong upstairs, weak downstairs."

35. D: 80–90 mg/kg/day divided BID *[Otitis media]*
High-dose amoxicillin for AOM differs from the GAS regimen (50 mg/kg once daily).

36. A: Place the infant in a knee-chest position *[Cyanotic CHD]*
Tet spell → knee-chest (raises systemic resistance and pushes blood to the lungs), calm the infant, give O2, call for help.

37. C: Wide, fixed split S2 *[ASD/VSD]*
ASD → "Always Split, Doesn't change." Increased right-sided flow delays pulmonic valve closure.

38. C: Risk of Reye syndrome *[Medications]*
Reye syndrome risk. Kawasaki disease is a specialist-directed exception.

39. A: Refer, because the passing threshold at age 4 is 20/40 *[Vision]*
AAPOS: age 3 = 20/50, age 4 = 20/40, age 5+ = 20/32. "Child grows, number shrinks."

40. D: Low-dose inhaled corticosteroid *[Asthma]*
Mild persistent = Step 2: daily low-dose ICS. "ICS = Insurance."

41. C: Hypertrophic cardiomyopathy *[Murmurs]*
HCM gets louder with standing/Valsalva (less filling). Most innocent murmurs get softer.

42. C: 1.2 million units *[Strep pharyngitis]*
27 kg is the dividing line: under 27 kg = 600,000 units; 27 kg or more = 1.2 million units.

43. C: Observe for 2–4 hours for return of symptoms *[Croup]*
Epinephrine works fast but wears off. "Watch 2–4 hours before the door."

44. C: Azithromycin *[Pneumonia]*
"My School = Mycoplasma." A gradual, atypical pattern in a school-age child is treated with a macrolide.

45. A: Coronary artery aneurysm *[Kawasaki]*
"Kawasaki Crowns the Coronaries." (Rheumatic fever targets valves.)

46. A: Yellow zone *[Asthma]*
180/300 = 60%. Green is 80% or more, yellow 50–79%, red under 50%.

47. D: Moderate to severe bulging of the tympanic membrane *[Otitis media]*
"Bulge = AOM. Red alone = Rubbish." Crying reddens the TM, and tugging is nonspecific.

48. D: Sweating and tiring during feeds, with poor weight gain *[CHD basics]*
"Feeding is the infant's treadmill." Peripheral edema is less prominent than in adults.

49. B: Chlamydia trachomatis *[Pneumonia]*
Chlamydia's 3 C's: Choppy (staccato) cough, Cool (afebrile), Conjunctivitis first. Classic at 1–3 months.

50. D: Reduces the risk of acute rheumatic fever *[Strep pharyngitis]*
"Penicillin protects the Heart, not the Kidney." Treatment prevents ARF; antibiotics do not reliably prevent PSGN.

51. C: Acute rheumatic fever *[Rheumatic fever]*
JONES: 2 majors (Joints + carditis) plus evidence of prior strep = ARF.

52. C: A single dose of oral dexamethasone *[Croup]*
"Dex for all." Mild croup = dexamethasone 0.6 mg/kg once. Croup is a clinical diagnosis.

53. B: Staphylococcus aureus, including MRSA *[Pneumonia]*
After the flu → think Staph (including MRSA) and pneumococcus.

54. A: Still give 2 routine MMR doses starting at 12 months *[Immunizations]*
A dose before 12 months does not count toward the routine 2-dose series.

55. C: Pain with eye movement and proptosis *[Eye]*
Orbital = 4 P's: Pain with movement, Proptosis, Poor vision, Paralysis (restricted movement) → emergency IV antibiotics.

56. C: Ventricular septal defect *[ASD/VSD]*
VSD = "Very loud." And a small VSD can be loud: "Small hole, loud howl."

57. D: Kawasaki disease *[Kawasaki]*
CRASH and BURN: fever 5+ days plus 4 of 5 (Conjunctivitis, Rash, Adenopathy, Strawberry tongue, Hands/feet).

58. B: The ears, for concurrent otitis media *[Eye]*
Bacterial conjunctivitis + AOM (often nontypeable H. flu) → amoxicillin-clavulanate.

59. D: Grade IV *[Murmurs]*
"4 = you can FEEL it." Grade III is loud without a thrill; VI is heard with the stethoscope off the chest.

60. D: All lesions are crusted over *[School and teaching]*
Varicella: all lesions crusted. A fixed number of days alone is not enough.

61. B: Holter monitor *[Cardiac tests]*
"Holter = Hold on for a day" (frequent symptoms). "Event = Every now and then" (rare symptoms).

62. D: Tdap, MenACWY, and HPV *[Immunizations]*
"Teens Meet HPV." HPV may start at age 9; MenACWY booster at 16.

63. A: 11 months *[Kawasaki]*
"2 grams → wait 11 months." Non-live vaccines do not need to wait.

64. B: Chalazion; warm compresses and lid hygiene *[Eye]*
"A Stye Stings, a Chalazion is Chill." A chalazion is sterile, so antibiotics are not routine. Never squeeze.

65. A: DTaP, IPV, Hib, PCV, and rotavirus *[Immunizations]*
2, 4, 6 months: "Dad Is Having Pizza Rolls" = DTaP, IPV, Hib, PCV, Rotavirus (plus HepB timing).

66. B: Tetralogy of Fallot *[CHD basics]*
Cyanotic = the 5 T's. The "D's" (ASD, VSD, PDA) are acyanotic left-to-right shunts.

67. A: 500 mg per dose, twice daily *[Dose calculation]*
"Per DOSE → Don't divide." 25 × 20 = 500 mg each dose (max 500 mg/dose).

68. D: Poststreptococcal glomerulonephritis *[Strep complications]*
PSGN = "Cola, Puffy, Pressure": dark urine, edema, and high BP after strep.

69. A: DTaP, IPV, MMR, and varicella *[Immunizations]*
School entry = "DIMV."

70. B: Give both vaccines as scheduled *[Immunizations]*
Household immunocompromise is not a reason to withhold MMR/varicella. If a varicella rash develops, avoid direct contact until it resolves.

71. B: A child with a prosthetic heart valve *[Other cardiac]*
Prophylaxis is limited to the highest-risk patients (prosthetic valve, prior IE, selected cyanotic/repaired CHD).

72. C: Increased left atrial pressure *[Fetal circulation]*
Lungs expand → PVR falls, cord clamping raises systemic resistance → left atrial pressure rises and pushes the flap closed.

73. B: Hearing loss from middle-ear fluid (OME) *[Ear and hearing]*
Conductive = Cerumen and fluid (outer/middle ear). Sensorineural = Snail (cochlea) and nerve.

74. B: High-dose amoxicillin, about 90 mg/kg/day divided BID *[Pneumonia]*
Typical bacterial pneumonia (S. pneumoniae) → amoxicillin. A macrolide is not an automatic substitute.

75. A: Right atrium to left atrium *[Fetal circulation]*
Foramen Ovale = "Open door between the atria" (RA → LA). Ductus arteriosus = PA → aorta. Ductus venosus bypasses the liver.

76. B: A button battery *[Ear and hearing]*
Never irrigate the 3 B's: Battery, Beans (vegetable matter), Broken eardrum. A battery requires urgent removal.

77. D: Still's (innocent) murmur *[Murmurs]*
Innocent = 7 S's. Still's is vibratory/musical and louder supine.

78. A: Rinse the mouth and spit after each use *[Asthma]*
Rinse and spit reduces thrush. ICS is used daily. Dry-powder inhalers do NOT use spacers.

79. B: MMR and varicella *[Immunizations]*
The "live birthday party" at 12–15 months: MMR + varicella (plus Hib/PCV boosters and HepA).

80. A: This is not contagious; use cool compresses and allergy eye drops *[Eye]*
Allergic = Itchy. Not contagious. Reduce exposure; antihistamine/mast-cell stabilizer drops.

81. D: Arthralgia cannot be counted when arthritis is already a major criterion *[Rheumatic fever]*
Don't double-count: arthritis + arthralgia, or carditis + prolonged PR.

82. C: Access-site bleeding and pulses distal to the site *[Cardiac tests]*
After a cath, monitor the site for bleeding and check distal pulses and perfusion.

83. C: Give the scheduled vaccines today *[Immunizations]*
Mild illness and antibiotics are not contraindications. Moderate/severe illness is a precaution.

84. B: 10.3 mL *[Dose calculation]*
22 × 15 = 330 mg. 330 × 5 ÷ 160 = 10.3 mL.

85. D: Supportive care; antibiotics are not routinely indicated *[Other respiratory]*
Acute bronchitis is usually viral. Purulent sputum alone does not justify antibiotics.

86. A: Indomethacin or ibuprofen *[PDA/obstructive]*
"Indomethacin Interrupts (closes); Prostaglandin keeps it Patent."

87. D: 900 mg *[Dose calculation]*
44 ÷ 2.2 = 20 kg. 20 × 90 = 1,800 mg/day ÷ 2 = 900 mg per dose. "Per DAY → Divide."

88. A: Another dose, because doses less than 5 months apart are not valid for the 2-dose series *[Immunizations]*
Started before 15 = 2 doses 6–12 months apart; if less than 5 months apart, a third dose is needed.

89. C: 9 mL *[Dose calculation]*
18 × 10 = 180 mg. 180 × 5 ÷ 100 = 9 mL.

90. D: Amoxicillin-clavulanate *[Otitis media]*
Augmentin 3 C's: amox in the last 30 days, Conjunctivitis plus otitis (nontypeable H. flu), Can't clear.

91. C: Paroxysmal *[Pertussis]*
"Cold → Cough → Calm" = Catarrhal → Paroxysmal → Convalescent.

92. D: Impending respiratory failure *[Asthma]*
"A silent chest is a scream." Less wheeze with worse air entry and fatigue means respiratory failure.

93. B: Do not start rotavirus; the first dose must be given by 14 weeks 6 days *[Immunizations]*
Rotavirus: start by 14 weeks 6 days ("by 15 weeks"), finish by 8 months.

94. C: It is not valid; injectable live vaccines must be given the same day or at least 28 days apart *[Immunizations]*
"Live shots: same day or wait a month (28 days)." The second live vaccine given too early should be repeated.

95. B: Pulmonary vascular resistance fell, increasing left-to-right shunting *[ASD/VSD]*
As pulmonary resistance drops over the first weeks, more blood crosses to the lungs → heart failure.

96. D: Prostaglandin E1 to keep the ductus open *[Cyanotic CHD]*
TGA = "Two separate circles" that need mixing. Never close that PDA.

97. D: IVIG 2 g/kg plus aspirin *[Kawasaki]*
IVIG within the first 10 days reduces coronary complications. Kawasaki is the exception to avoiding aspirin.

98. C: Daytime cough and nasal drainage for more than 10 days without improvement *[Other respiratory]*
"10 days, Double-dip, 39 for 3." Colored mucus alone is not enough.

99. D: Cough, rhinorrhea, hoarseness, and oral ulcers *[Strep pharyngitis]*
"Looks like a COLD → not strep." Exudate alone does not prove GAS because EBV and other viruses can cause it.

100. C: Watchful waiting for about 3 months, with a hearing check if it persists *[Ear and hearing]*
OME = "Fluid without Fire." No antibiotics, antihistamines, decongestants, or steroids.

Score yourself by topic

Count your misses per topic tag in the answer key. Any topic with 2+ misses: reread that section of the mnemonics guide, then redo only those questions tomorrow.